

Clinician Conversation Guide

Ovarian cancer detection in primary care: explaining screening absence, responding to patient concerns, and supporting timely investigation

1. Why no routine screening

2. Explaining it to patients

3. Responding to CA-125 requests

4. Investigating symptoms

5. Managing uncertainty

6. Referral pathway

Why no routine ovarian cancer screening exists

Understanding the evidence base helps you explain it clearly and confidently to patients.

UKCTOCS trial (Menon et al., 2023)

The largest ovarian cancer screening RCT to date, involving over 200,000 women showed that neither annual CA-125 monitoring nor transvaginal ultrasound screening reduced ovarian cancer mortality in average-risk, asymptomatic women over 16 years of follow-up.

Harms of false positives

CA-125 and ultrasound screening generate false-positive results that can lead to unnecessary surgical intervention, procedural complications, and significant patient anxiety, without a corresponding mortality benefit in asymptomatic populations.

Guideline consensus

ACOG, NICE, Cancer Care Ontario, and the Canadian Task Force on Preventive Health Care all recommend against routine ovarian cancer screening in average-risk asymptomatic women. This is not a gap: it reflects the weight of global evidence.

Key clinical distinction: CA-125 and transvaginal ultrasound are appropriate and evidence-supported as diagnostic tools when a patient presents with symptoms. They are not appropriate as population screening tests in asymptomatic, average-risk women.

Note for clinicians: Stewart et al. (2012) found that that most primary care physicians incorrectly believed CA-125 to be an effective screening tool for asymptomatic women. Reviewing this evidence supports both personal clinical practice and patient communication.

Explaining the absence of screening to patients

Patients often feel alarmed when told there is no routine screening. The goal is to explain the evidence clearly, acknowledge their concern, and redirect toward symptom awareness without creating false reassurance.

SUGGESTED LANGUAGE - EXPLAINING NO ROUTINE SCREENING

"Right now, there is no routine blood test or scan that reliably detects ovarian cancer in women who have no symptoms. This is not a gap in care; large clinical trials have tested these approaches and found they do not reduce ovarian cancer deaths in women who do not experience symptoms, and they can sometimes lead to unnecessary procedures. What we do know is that ovarian cancer does cause symptoms, and recognizing those symptoms early and coming in promptly makes a real difference."

AVOID

"There is nothing we can do to check for ovarian cancer" overstates the situation and causes alarm

"You are fine, your pelvic exam was normal" creates false reassurance about ovarian status

"We only screen high-risk patients" may suggest screening exists when it does not

WHAT WORKS WELL

Name the evidence directly. Patients find "large clinical trials" more reassuring than vague statements

Pair the absence of screening with a clear action: symptom awareness and prompt help-seeking

Validate the concern before correcting the misconception

Responding to CA-125 requests and Pap test misconceptions

When a patient asks for CA-125 testing

This is a common and understandable request. The goal is to explain the limitation without dismissing the concern.

SUGGESTED RESPONSE

"CA-125 is a blood test we use when someone already has symptoms that concern us. In women without symptoms, it produces a lot of false alarms and can lead to procedures you do not need. That is why it is not recommended as a routine screening test. I want to make sure that you know what symptoms to watch for so you can come in promptly if something feels off."

When CA-125 is appropriate

Persistent or frequent symptoms (bloating, pelvic pain, early satiety, urinary changes) that are new or different from baseline, especially occurring 12 or more times per month.

When a patient believes her Pap test screens for ovarian cancer

Multiple studies document this misconception. It is very common not a patient failing.

SUGGESTED RESPONSE

"I am really glad you mentioned that, because it is a very common misunderstanding. The Pap test only looks at cells from your cervix; it screens specifically for cervical cancer. It does not look at your ovaries at all. So, a normal Pap result is good news for your cervix, but it does not tell us anything about your ovaries."

Why this correction matters

Women who believe their Pap test screens for ovarian cancer may experience false reassurance after a normal result, making them less likely to report persistent symptoms or seek care promptly. Correcting this directly, without embarrassing the patient is one of the highest-value communication acts in this space.

Investigating symptoms: when and how to act

Because there is no screening, the primary care encounter is the critical point in the diagnostic pathway. Consistent, timely response to symptoms is where outcomes are most likely to improve.

THE SYMPTOM INDEX - investigation threshold

Bloating	New, frequent, persistent
Pelvic or abdominal pain	Including pressure or discomfort
Early satiety or difficulty eating	Feeling full quickly
Urinary urgency or frequency	New change from baseline
Unexplained weight loss	Or abdominal swelling
Change in bowel habits	Not explained by diet change

Investigate if symptoms occur 12 or more times per month AND are new or represent a change from the patient's baseline.

Initial investigation steps

- 1 Take a symptom history**
 Duration, frequency, change from baseline. Ask specifically about bloating, pelvic pain, satiety, and urinary changes.
- 2 Pelvic examination**
 Including bimanual and speculum examination to assess for adnexal mass or abnormality.
- 3 CA-125 blood test**
 Order alongside pelvic ultrasound when symptoms meet the investigation threshold.
- 4 Transvaginal ultrasound**
 Preferred over transabdominal. If result is abnormal or CA-125 is elevated, refer urgently.
- 5 Document and follow up**
 If first investigation is negative but symptoms persist, reassess rather than discharge. **Do not offer reassurance alone in response to ongoing symptoms.**

Managing uncertainty over time and the referral pathway

Managing uncertainty without false reassurance

Studies show that patients sometimes feel dismissed at early healthcare encounters, and that symptom normalization or premature reassurance contributes to delayed diagnosis. These scripts support a better approach.

WHEN INITIAL INVESTIGATION IS NEGATIVE

"The results today are reassuring, but I do not want to just leave it there. If these symptoms continue or get worse over the next few weeks, I want you to come back in. We would look at this again rather than assuming everything is fine."

WHEN A PATIENT FEELS THEIR SYMPTOMS ARE BEING DISMISSED

"I hear you, and I want to take this seriously. You know your body better than anyone. Let us go through what you have been experiencing in detail, and we will make sure we have done a thorough assessment."

Core principle

Reassurance alone is not an appropriate response to persistent or worsening symptoms. Document the symptom pattern, set a clear review date, and specify the threshold for further action.

Referral pathway

Urgent referral to gynecology

- ↘ Elevated CA-125 with pelvic symptoms
- ↘ Adnexal mass or pelvic abnormality on exam
- ↘ Ascites or unexplained abdominal swelling
- ↘ Strong family history plus new pelvic symptoms

Non-urgent referral or repeat assessment

- ↘ Persistent symptoms after initial negative investigation
- ↘ Patient anxiety warrants specialist reassurance
- ↘ Symptoms not clearly meeting threshold but recurrent

Manage in primary care with follow-up plan

↘ Initial investigation negative and symptoms resolving

↘ Document and set clear return threshold for the patient

Ovarian cancer in primary care: the key points at a glance

The evidence base

- ✖ No population screening test is recommended for average-risk asymptomatic women (ACOG, NICE, UKCTOCS)
- ✖ CA-125 and TVUS are diagnostic tools, not screening tools
- ✖ Many physicians incorrectly believe CA-125 is a valid screening test. Check your own assumptions
- ✖ Pap test screens for cervical cancer only, it provides no information about the ovaries

When to investigate

- ✖ Symptoms new or different from baseline
- ✖ Occurring 12 or more times per month
- ✖ Bloating, pelvic pain, early satiety, urinary urgency or frequency
- ✖ Investigate with: pelvic exam, CA-125, transvaginal ultrasound
- ✖ If negative but symptoms persist: reassess, do not just reassure

What to say

- ✖ "There is no reliable screening test, this is based on evidence, not a gap in care"
- ✖ "Your Pap test was for your cervix, it tells us nothing about your ovaries"
- ✖ "CA-125 is for investigating symptoms, not for routine checks"
- ✖ "If these symptoms continue, come back we will look at this again"
- ✖ Validate concern before correcting the misconception